3.9 Pneumothorax and Chest Drain

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I felt sudden chest pain and could not breathe." **Objective** • No breath sounds on one side • Chest moves unevenly • SpO_2 88%, RR 30/min	**ACTUAL** **Ineffective breathing pattern** related to lung collapse as evidenced by dyspnea, absent breath sounds and hypoxemia.	Within 24 hours the lung has re-expanded and SpO_2 is 94% or more.	• Give oxygen as prescribed. • Assist with putting in the chest drain. • Keep the drainage system below chest level and the tubing free of kinks. • Look for swinging (tidaling) and bubbling in the water seal. • Tracheal deviation, low BP or worsening breathlessness must be reported at once (tension pneumothorax).	Lung re-expanded on X-ray, SpO_2 on target: **goal met**.
Subjective • "The tube hurts when I move." **Objective** • Guards the drain site • Does not want to move	**ACTUAL** **Acute pain** related to chest drain and pleural irritation as evidenced by guarding and reported pain.	Pain 3/10 or less and doing his breathing exercises.	• Give analgesia regularly as prescribed. • Support the tube so it does not pull. • Encourage deep breathing and exercises for the arm on the affected side.	Does the exercises with pain he can tolerate: **goal met**.
Subjective • *No specific complaint* **Objective** • Chest drain in situ	**RISK** **Risk for injury** as evidenced by risk factors: accidental disconnection or dislodgement of chest drain.	Drain stays secure and working.	• Make sure every connection is secure. • Never clamp the drain unless it is ordered. • Keep sterile gauze and clamps at the bedside. • If the tube falls out, cover the site with an occlusive dressing and call the physician immediately.	Drain not dislodged: **goal met**.